Entrectinib monotherapy · adult

Adult treatment scope · NCCP 00702a / 00702b This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous entrectinib · individual support

Active medicines: entrectinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- National Cancer Control Programme (NCCP) minimal-to-low oral emetic risk: agree individual rescue treatment and check QT/ interactions. NCIS V8 parenteral schedules are not a daily oral prescription.
- No routine premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), steroid course or antimicrobial prophylaxis is specified.
- Give the team a complete medicine and supplement list, including St John's wort. CYP3A inhibitors/inducers and medicines affecting heart rhythm need pharmacy review. Do not change the cancer dose yourself; temporary interaction doses differ from toxicity reductions.
- Take whole capsules with or without food; do not crush or chew. Avoid grapefruit, grapefruit products and Seville oranges. If a dose is missed, take it unless the next dose is due within 12 hours; confirm the exact boundary with your pharmacist. The leaflet permits repeating whole capsules after immediate vomiting: follow your written plan and call if uncertain. Suspension or tube administration requires separate pharmacy instructions.
- Avoid pregnancy. Women who could become pregnant need highly effective contraception during treatment and for at least five weeks afterwards; add a barrier method if using systemic hormonal contraception. Men with partners who could become pregnant need highly effective contraception during treatment and for at least three months afterwards. Do not breastfeed during treatment. Discuss fertility and family plans before starting.
- Review cognition/driving, falls/fractures, uric acid, swelling, visual symptoms and practical adherence needs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00702 v2 · p6–7 · NCCP 00702 v2; oral emesis guidance V6 and NCIS V8 scope